



## POLICY & PROCEDURE

### Medication: Patient's Own Medications

|                       |                                                                           |
|-----------------------|---------------------------------------------------------------------------|
| <b>Scope (Staff):</b> | North Metropolitan Health Service Mental Health (NMHS MH): Clinical Staff |
| <b>Scope (Area):</b>  | NMHS MH Inpatient Sites                                                   |

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## 1. Aim

To ensure the safe management and storage of Patient's Own Medications (POMs) for the patient whilst in hospital.

## 2. Background

POMs refer to all prescribed and non-prescribed medication brought into hospital by the patient, patient's family, carer, personal support person, Hospital in the Home (HiTH) staff members, ambulance officers or police officers. POMs are considered patient's property.

Patients are encouraged to bring their own medications to hospital to aid clinical staff obtaining a Best Possible Medication History for Medication Reconciliation.

POMs brought into NMHS MH sites will be recorded and stored in accordance with relevant legislation, and returned to the patient on discharge when it is appropriate to do so; as per the [WA Medicine Handling Policy](#); [WA Medicines & Poisons Regulations 2016](#); [WA Storage of Medicines and Poisons](#) and the [NMHS Medicines Management Policies and Procedures](#).

When POMs that have been brought into hospital are deemed unsuitable for returning to the patient on discharge this will be documented in the patient's Integrated Progress Notes (PMR 7) and discussed with the patient, patient's family, personal support or nominated person where appropriate.

## 3. Risk

To avoid the risk of deliberately or inadvertently mishandling or loss of POMs whilst in hospital.

## 4. Definitions

|                                        |                                                                                                                                                                                                                                                                                 |
|----------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Patients Own Medications (POMs)</b> | ALL prescribed and non-prescribed medication brought into hospital by the patient, patient's family, carer or other personnel.                                                                                                                                                  |
| <b>Controlled Substance</b>            | This term includes both Schedule 8 (S8) and (drugs of addiction; drugs of dependence) Schedule 4 Restricted (S4R) Medicines (considered to have a higher risk of diversion).                                                                                                    |
| <b>Schedule 8 (S8)</b>                 | Medicines with a very high risk of diversion and misuse. Also referred to as 'drugs of addiction', 'controlled medicines' or 'drugs of dependence'. The list of medicines designated S8 are listed in the Standard for the Uniform Scheduling of Medicines and Poisons (SUSMP). |
| <b>Schedule 4 Restricted (S4R)</b>     | These are Schedule 4 (S4) medicines considered to have a higher risk of diversion and misuse. Also referred to as 'controlled medicines'. The list of medicines designated S4R are listed in the Risk Based Requirements for Medicines Handling policy                          |
| <b>Approved Staff Member</b>           | Refers to a NMHS staff member who is: <ul style="list-style-type: none"><li>• an authorised staff member, or</li><li>• a staff member who is allocated the responsibility to complete a particular task related to medicines as per their job description.</li></ul>            |

|                                                         |                                                                                                                                                                                                                                                                                                                                                                                                                                      |
|---------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Authorised Staff Member</b>                          | A health practitioner who is a NMHS employee authorised by legislation to be involved in certain processes such as procurement, distribution, supply, storage, administration and/or disposal of scheduled medicines. The processes a practitioner can participate in are dependent on their individual profession and any additional training or endorsement they hold as defined in the WA Medicines and Poisons Regulations 2016. |
| <b>Chain of Custody</b>                                 | In relation to medicines, the chronological documentation that records the transfer of a medicine from one individual/area to another. Sometimes described as the “paper trail”, even though the records may be electronically made and stored. A critical element of maintaining the chain of custody is that the records must be robust and auditable.                                                                             |
| <b>Statement of Patients Own Medication (SPOM) book</b> | A patient’s medication property book to list all medication brought into hospital belonging to the patient.                                                                                                                                                                                                                                                                                                                          |
| <b>Patient’s Own Medication Bag</b>                     | Bag for all medications except S4R/S8 medications. <a href="#">Appendix 3</a> .                                                                                                                                                                                                                                                                                                                                                      |
| <b>Tamper-evident S4R or S8 POM Bag</b>                 | Secure, tamper-evident medication bag with a unique serial number. Refer to <a href="#">Appendix 2</a> .                                                                                                                                                                                                                                                                                                                             |
| <b>Dose Administration Aids (DAA)</b>                   | Webster pack, dosette box, sachets                                                                                                                                                                                                                                                                                                                                                                                                   |

## 5. Principles

### 5.1 Receiving, documenting and storing POMs

1. All medications brought into the hospital by patients must be checked and documented on the SPOM form by TWO approved staff members.
2. Loose tablets or capsules which cannot be identified will be recorded as unknown medication, seek advice from a clinical pharmacist if necessary.
3. The pink copy of the SPOM form must be filed in the patient’s medical record. The blue copy of the SPOM form must be given to the patient/ patient’s family/ personal support or nominated person.
4. The white copy of the SPOM form and the medications must then be placed in a green POM bag, except for any S8 or S4R medication (refer to Section 5.2), which must be stored in the appropriate safe or locked cupboard. The bag must be labelled with an addressograph and stored securely as per local site procedure:
  - Graylands Hospital, Frankland Centre and Selby Lodge – all POMs will be sent to the pharmacy department for storage. POMs will be given to the clinical pharmacist,



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imprest pharmacy assistant or taken to pharmacy as soon as possible after the patient is admitted to the ward.

- Osborne Lodge – POMs stored in the medication room.
  - Sir Charles Gairdner Hospital Mental Health Unit (SCGH MHU) – POMs stored in MHU Medication Room.
  - Sir Charles Gairdner Hospital Mental Health Observation Area (SCGH MHOA) – POMs stored in the bottom drawer of the medication trolley.
5. Refrigerated medications must be placed in the ward medication refrigerator and location of item must be clearly documented on the SPOM form.
  6. When POMs are sent to pharmacy, SPOM form, a completed requisition form and documentation out of the ward register is required for S4R and S8 medications.

## 5.2 Receiving, documenting and storing S8/S4R POMs

1. POMs containing S8/S4R medicines require more vigorous documentation and storage to prevent diversion. The chain of custody must be maintained from admission, transfer, return to patient or destruction, as with all other S8/S4R medicines.
2. All S8 medicines brought into the hospital by patients must be documented on a SPOM form and in the back of the S8 'Drugs of Addiction' register by two approved staff members.
3. All S4R medicines must be documented on a SPOM form and in the back of the S4R register by two approved staff members
4. Once a physical count is done by approved staff, all S8 and S4R medication must be placed in a tamper-evident Patient's S8 or S4R Medication Bag ([Appendix 2](#)) and sealed, then stored in the appropriate S8 safe or S4R cupboard on the ward.
5. The location, number of S8 and S4R bags used and the unique serial number of tamper-evident medication bag(s) must be indicated on the SPOM form.
6. The POM must be recorded in the appropriate register, one patient per page with patient name, UMRN and date of birth or an addressograph. The name, strength, form and quantity of the S8/S4R medication, must be recorded together with the date, time of entry and the unique serial number of the tamper-evident POM bag.
7. Where there are other medications and S8/S4R medicines, two SPOM forms must be written, one for the other medications and one for the S8/S4R medicines. The SPOM for the S8/S4R medicine must be placed in the S8/S4R tamper-evident medication bag, with the list of medicines visible through the bag. The SPOM form for the other medication must be placed in the green POM bag with the medication.
8. Refer to [NMHS Management of Patients' Own Schedule 8 \(S8\) and Schedule 4 Restricted \(S4R\) Medicines](#).



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### 5.3 Documentation of POMs in Dose Administration Aids (DAAs)

1. There are various types of DAAs available. The most common types are blister-packs (e.g. Webster-paks®, sachets and dosette boxes).
2. In some cases, medications in the DAA may not be identifiable due to various reasons, e.g. Webster-pak® header removed, packed in an unlabelled dosette box, no markings, etc. The patient, patient's family or carer, GP, community pharmacy or MIMS (reference) may be able to provide this information, or the clinical pharmacist can advise during the next working day.
3. Unidentifiable medicines are recorded as unknown/ unidentified medicines. These may not be suitable to return to the patient due to storage conditions and stability of medicines once removed from original packaging. A discussion with the team, clinical pharmacist and patient, patient's family, personal support or nominated person to destroy/dispose of the medications is required and must be documented on the SPOM form.
4. The DAA must be documented on the SPOM form as (X number) DAAs containing (Y number doses), each dose being a complete sealed blister, sachet or a compartment in a dosette box.
5. For POM oral liquids an estimate of the quantity left in the container is acceptable. Mark the level of liquid on the container. Where possible the patient should endorse the volume recorded on the SPOM form.

### 5.4 DAA POMs containing S8/S4R Medicines

1. S8 and S4R medicines in DAAs require documentation on the SPOM form stating the name of S8/S4R medicines contained in the DAA. They must also be recorded in the appropriate register.
2. If packs contain both S8 and S4R medicines, the entire DAA is treated as a S8 POM; if packs contain S4R medicines the entire DAA is treated as a S4R POM. If packs contain S8 medicines, the entire DAA is treated as a S8 POM. The SPOM form must document where the DAA is stored.
3. In relation to blister-packs, staff entering medication(s) into the register, document the contents of the pack based on the written description of the pack contents and how many doses remain intact based on visual inspection of the pack (e.g. tramadol stated to be packed as one 50mg tablet twice daily, 10 of the 14 dose times are intact/sealed/unbroken, therefore a quantity of 20 tablets is entered into the register).
4. Where possible, use the larger sized tamper-evident medication bag. The extra space inside the bag may minimise the obscuring of details on the DAA, this allows inspection of details on the pack without having to open the bag.
5. Place the white SPOM form in the bag with the medication list visible through the bag.

### 5.5 Other information regarding POMs containing S8/S4R Medicines

1. At no point will the S8 or S4R medication be removed from the tamper-evident medication bag once it is sealed, unless a clinical decision has been made to only give a partial return of medication, or there is a need to use the patient's own S8 or S4R medication on the ward.



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2. When S8 or S4R medicine is used for administration during admission, a new tamper-evident bag will be used each time a dose is given. The contents will be checked, and the new unique serial number will be documented.
  3. Do not place individual S8 or S4R medicines into the same bag. If a DAA contains both S8 and S4R medicines, the entire DAA is treated as a S8 POM.
  4. Whilst the POM containing S8/S4Rs is on the ward, the tamper-evident POM bag/s must be checked at least once a day (at the same time the ward S8/S4Rs are checked) to ensure the seal is intact and documented in the relevant register as checked and correct.

## 5.6 Transporting the S8/S4R POMs from the ward

### 5.6.1 Graylands Hospital, Frankland Centre and Selby Lodge

1. The clinical pharmacist will be contacted at the first opportunity to arrange removal of patient's own S8/ S4R medications from the ward.
2. A requisition form, either S8 or S4R as appropriate ([Appendix 4](#)) is required.
3. Medication can be returned to the pharmacy by authorised pharmacy staff.
4. Alternatively, the medication can be returned to the pharmacy by authorised ward staff (required to bring the ward register that the POM is documented in and medication to the pharmacy). The pharmacy personnel will then sign the medication out of the ward register and into the pharmacy POM register, including the requisition number. Both registers are signed by the authorised ward staff nurse and pharmacy personnel.

### 5.6.2 SCGH MHU, SCGH MHOA and Osborne Lodge

1. All POMs are kept on the ward until the patient is transferred or discharged.
2. All POMs containing S8 and S4R medications are locked in the appropriate S8 safe or S4R locked cupboard on the ward.

Refer to [NMHS Transport of Schedule 8 \(S8\) and Schedule 4 Restricted \(S4R\) Medicines](#)

### 5.6.3 Inter-hospital transfer of POMs

1. Two approved staff members must sign the S8/S4R POM out of the appropriate register, document the hospital the patient is being transferred to.
2. An authorised staff member must communicate with the receiving hospital that S8/S4R POMs are being transferred with the patient. Document in patient's integrated progress notes (NMHS MH SPOM form).
3. Refer to Section 5.4.



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## 5.7 Medication Reconciliation on admission

The POMs are checked at admission as part of the medication reconciliation process by the admitting doctor and clinical pharmacist.

## 5.8 Exceptions for using POMs on the ward

POMs will not be administered to the patient whilst in hospital, except:

- On authority of the doctor or clinical pharmacist when a medication is not available for immediate use, authority will be documented in the patient's integrated progress notes (PMR 7).
- Complementary or Alternative Medicines (CAM) will not be supplied by the hospital and where the treating doctor authorises treatment with CAM to continue, the POM may continue to be used.

These medicines must be prescribed on the WA Hospital Medication Chart (PMR 60A) as a POM.

### 5.8.1 Adrenaline (epinephrine) Auto-injectors

- Patient's own adrenaline (epinephrine) auto-injectors (Epipen®, Anapen®) auto-injector will be labelled with an addressograph and kept on the ward in the medication trolley.
- These are not to be used for allergic reactions on the ward, which must be managed as a Code Blue Medical Emergency.
- If the auto-injectors are in an appropriate condition, they must be given back to the patient when they leave the ward for ground access, go on hospital leave or are discharged from hospital.

## 5.9 Return of POMs on discharge or ward transfer

1. At discharge or ward transfer the nurse must check the pink copy of the SPOM in the patient's medical record to determine whether the patient has brought medications into hospital and contact the clinical pharmacist.
2. The clinical pharmacist, in consultation with the doctor, will review which medications are deemed appropriate to be returned to the patient, carer or transferred with the patient to the new care facility. The pharmacist must record on the pharmacy copy of the SPOM form which medications are to be returned to the patient.
3. The patient or carer will sign under the 'The Above Specified Items Have Been Returned to Me' section on the pink copy in the patient's medical record or white copy in pharmacy of the SPOM form, when medications are returned.
4. The pink SPOM form is filed in the patient's medical record; the white SPOM form in the pharmacy with the POM until the patient is discharged, then filed in discharge file.



## 5.10 Destruction of expired, unwanted or returned POMs

1. The medical officer and clinical pharmacist may decide at the time of the admission, that the POMs are unsuitable to be returned to the patient and can be destroyed immediately. This must be documented on the SPOM form. DAAs are generally destroyed immediately after medication reconciliation.
2. As POMs are recognised as property of the patient, consent is required from the patient/carer to dispose any POMs. This must be documented on the SPOM form prior to discharge.
3. If the patient does not consent to destruction of POMs but the treating team believe it is not clinically appropriate for the medicine to be returned to the patient, the doctor must document authority to destroy the medicines in the patient's integrated progress notes.
4. Prior to discharge, the patient's clinical pharmacist must document on the SPOM form if any remaining POMs are to be returned to the patient.
5. Any expired, unwanted or unreturned POMs will be destroyed by pharmacy after a minimum of four weeks following the patient's discharge. The destruction of POMs must be documented on the white pharmacy copy of the SPOM form.
6. S4R and S8 medications must be destroyed as outlined in the [NMHS Destruction, Discard and Disposal of Schedule 8 \(S8\) and Schedule 4 Restricted \(S4R\) Medicines procedure](#).

## 5.11 Broken seal of tamper-evident medication bag or missing bag

Where the seal has been broken for no legitimate reason, the bag is missing, or for any other S8 or S4R discrepancies, follow the protocol for [NMHS Discrepancy Management of Schedule 8 \(S8\) and Schedule 4 Restricted \(S4R\) Medicines](#) and [NMHS MH Medication: Management of Controlled \(Schedule 8 \(S8\) and Schedule 4 Restricted \(S4R\)\) Medicines](#).

## 6. Compliance and Evaluation

This policy will be audited annually by NMHS MH Pharmacy and reported to NMHS MH DTC and NMHS MH Clinical Governance Committee.

- 100% of POMs containing controlled substances will be recorded in registers and stored in tamper-evident bags.
- 100% of POMs will be documented on a SPOM form.
- 100% of SPOM will record decision to return to patient or destroy.

### Related internal policies, procedures and guidelines

[NMHS MH Medication: Management of Controlled \(S8 & S4R\) Medicines](#)

[NMHS Destruction, Discard and Disposal of Schedule 8 \(S8\) and Schedule 4 Restricted \(S4R\) Medicines Policy](#)

[NMHS Destruction, Discard and Disposal of Schedule 8 \(S8\) and Schedule 4 Restricted \(S4R\) Medicines Procedure](#)

[NMHS Management of Patients' Own Schedule 8 \(S8\) and Schedule 4 Restricted \(S4R\) Medicines](#)



[NMHS Transport of Schedule 8 \(S8\) and Schedule 4 Restricted \(S4R\) Medicines](#)  
[NMHS Discrepancy Management of Schedule 8 \(S8\) and Schedule 4 Restricted \(S4R\) Medicines](#)

## References

[WA Medicines & Poisons Regulations 2016](#)

[WA Medicines & Poisons Act 2014](#)

[WA Medicines Handling Policy](#)

[WA Guideline on patients' own medicines](#)

|                                                                                            |                                                                                                                                                                                                                                                                                                                |                |                                                                                                                                                  |              |            |
|--------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|----------------|--------------------------------------------------------------------------------------------------------------------------------------------------|--------------|------------|
| Policy Sponsor                                                                             | NMHS MH Chief Pharmacist                                                                                                                                                                                                                                                                                       |                |                                                                                                                                                  |              |            |
| Policy Contact                                                                             | NMHS MH Chief Pharmacist                                                                                                                                                                                                                                                                                       |                |                                                                                                                                                  |              |            |
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| Endorsed by:                                                                               | MHPHDS Executive Director                                                                                                                                                                                                                                                                                      |                |                                                                                                                                                  | Date:        | 23/01/2024 |
| NSQHS Standards Applicable:                                                                | <input checked="" type="checkbox"/>  Std 1: Clinical Governance<br><input checked="" type="checkbox"/>  Std 2: Partnering with Consumers |                | <input checked="" type="checkbox"/>  Std 4: Medication Safety |              |            |
| National Standards for MHS                                                                 | <input checked="" type="checkbox"/> Std 8: Governance, leadership and management                                                                                                                                                                                                                               |                | <input checked="" type="checkbox"/> Std 10: Delivery of Care                                                                                     |              |            |
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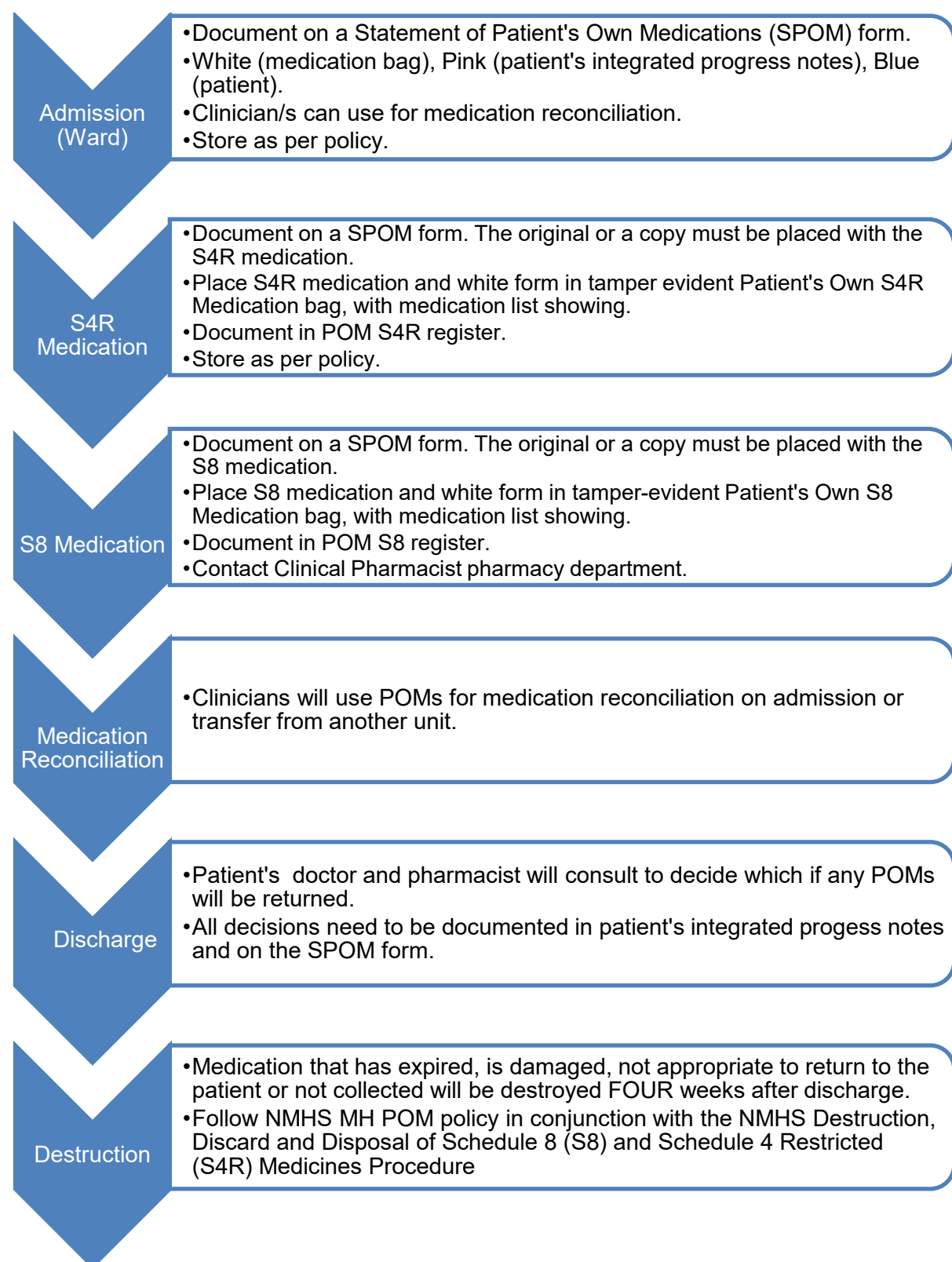
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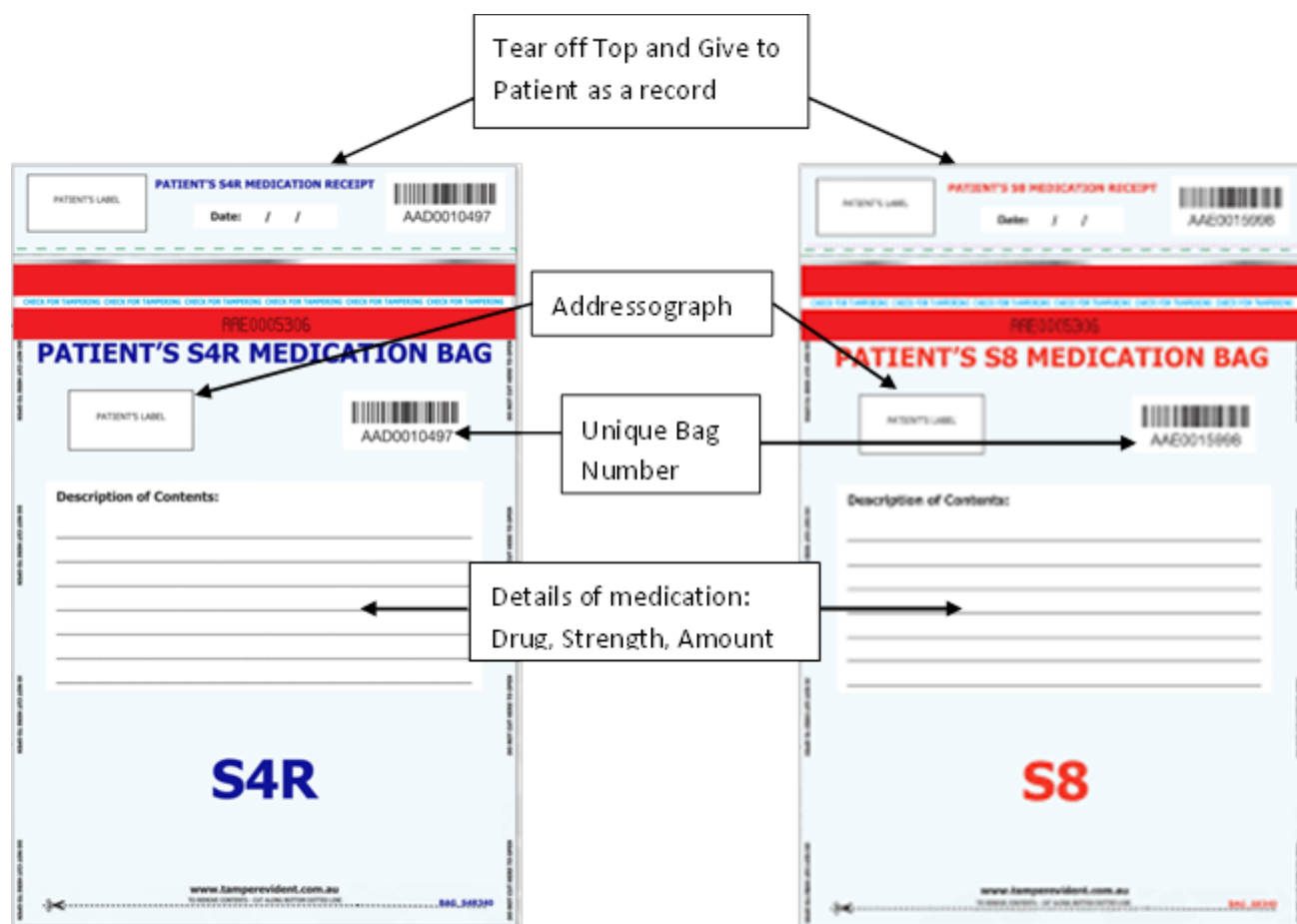
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## Appendix 1. Procedure Overview



## Appendix 2. S4R and S8 POM Tamper-evident Medication Bags



|         |                                                                                                                                                   |         |            |
|---------|---------------------------------------------------------------------------------------------------------------------------------------------------|---------|------------|
| 154412A | Bag, Plastic: Flat; "Patient's Own Medication"; S4R<br>Large; Tamper Evident; Tear Off Receipt; Barcode & Sequential Number; 75 Micron Co-Ex Film | Box 500 | BAG_S4R340 |
| 154413B | Bag, Plastic: Flat; "Patient's Own Medication"; S4R<br>Small; Tamper Evident; Tear Off Receipt; Barcode & Sequential Number; 75 Micron Co-Ex Film | Box 500 | BAG_S4R260 |
| 154411Y | Bag, Plastic: Flat; "Patient's Own Medication"; S8<br>Large; Tamper Evident; Tear Off Receipt; Barcode & Sequential Number; 75 Micron Co-Ex Film  | Box 500 | BAG_S8340  |
| 154414C | Bag, Plastic: Flat; "Patient's Own Medication"; S8<br>Small; Tamper Evident; Tear Off Receipt; Barcode & Sequential Number; 75 Micron Co-Ex Film  | Box 500 | BAG_S8260  |

## Appendix 3. Patient's Own Medication Bag



### Patient's Own Medication Bag With Tear Off Receipt

Document all medicines when a patient arrives at a hospital/health service and ensure that medicines are reconciled and returned correctly to the patient where appropriate with these Patient's Own Medication Bags.

#### TEAR OFF RECEIPT

For POM's stored on the ward document all patients' medications on the "Tear off Receipt" located at the top of the bag and store securely for the pharmacist or other appropriately skilled health professional to reconcile and return to the patient if appropriate.

#### BAG \_\_ OF \_\_

In the event more than one bag is used for a patient clearly number them accordingly here. e.g. Bag 1 of 2, 2 of 2.

#### PATIENT'S NAME/UMRN

Register patient's name by applying a label and Hospital Unit Medical Record Number (UMRN) that may assist hospitals to locate patient information/records.

#### DATE DISCHARGED TABLE

Record all other important information relevant to the patient here.

#### OTHER POM

Clearly mark medicines with specific storage instructions such as Schedule 8, Schedule 4 Declared Medicines and Fridge items such as eye drops.

The diagram shows a green and white medication bag. At the top, there is a section for 'PATIENT'S OWN MEDICATIONS stored on ward' with fields for 'Hospital Name', 'Patient addressograph', and 'Medication List'. Below this is a large green area with a white label that reads 'PATIENT'S OWN MEDICATIONS:' and 'PATIENT'S NAME/UMRN (ADDRESSOGRAPH)'. To the right of this label is a 'DATE DISCHARGED TABLE' with columns for 'Date discharged', 'Telephone Number', 'Phone call to pick up', 'Will collect on', and 'For destruction'. At the bottom left, there is a section for 'OTHER POM' with checkboxes for 'SAR', 'SB', and 'Fridge'.

MS100

Patient's Own Medication Bag  
With Tear Off Receipt  
350 x 430mm + 80mm receipt  
500 per box  
\$245.00

Available from Stirling Fildes PTY LTD

| Item Number | Item Description                                                                                                                                                                                         | Vendor Product Num | Unit of Purchase | Unit Price |
|-------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------------|------------------|------------|
| 201574G     | Bag, Plastic: Flat; 'Patient's Own Medication Bag'; with tear off receipt; Green/white; 430MM deep x 350MM + 800MM Receipt; Bag of; Patients Name/UMRN; Date Discharged Table; Other POM; Resealable bag | MS 100             | Box 500          | \$245.00   |

## Appendix 4. S8 and S4R Requisition Forms

### S8 Requisition Form

**HEALTH DEPARTMENT OF WESTERN AUSTRALIA**  
**REQUISITION FOR SCHEDULE EIGHT DRUGS** **D 015922** HA 219 07/11

Please supply the undermentioned Schedule Eight Drugs to  
Ward/Hospital

Quantity on Hand    Quantity Required    Item Description    Quantity Supplied    Pharmacy Use Only

Ordered By ..... Date .....  
(Signature of Nurse and name on back letters)

Supplied By ..... Date .....  
(Pharmacist's Signature)

Received By ..... Date .....  
(Nurse's Signature)

Details of Medication and Unique Bag

Sign Out of Ward by a Registered Nurse and Pharmacist

Requisition Number

### S4R Requisition Form

**NORTH METROPOLITAN AREA HEALTH SERVICE MENTAL HEALTH** ORIGINAL - BLUE  
DUPLICATE - WHITE

Requisition for **RECORDABLE SCHEDULE 4 DRUGS (S4R)**

Ward or Department ..... **5251**

Name of Preparation    Strength    Quantity or Number of Doses

Ordered by ..... Date .....  
(signature and print name)

Supplied by ..... Date .....  
(signature and print name)

Received by ..... Date .....  
(signature and print name)

Details of Medication and Unique Bag

Sign Out of Ward by an Authorised Nurse and one Pharmacy Staff

Requisition Number